



March 17th, 2025

RE:

- Patient Name: Jennifer Pei Ru Hao
- Date of Birth: August 13th, 1985

Meeting Mar 17, 2026 at 11:41 PDT

Meeting records File

Summary

Initial assessment confirmed a progression of central and peripheral vestibular issues, with a 6-month treatment plan developed, including specific BPPV maneuvers and 3PD optogenetics.

Vestibular Condition Progression Confirmed

Diagnosis confirmed the progression of vestibular issues, noting a worsening of findings including nystagmus in the dark and gaze-evoked nystagmus, confirming both 3PD and BPPV conditions. Spontaneous nystagmus was identified as the most significant physiological finding that requires elimination through treatment.

6-Month Treatment Plan Established

A complicated 6-month treatment timeline requiring twice-weekly appointments was estimated for effective recovery, following a tiered approach starting with BPPV maneuvers and progressing to slow optogenetics for 3PD. The clinician expressed high confidence that treatments will improve the condition, with the extent of recovery depending on treatment response.

Billing and Referral Logistics

The clinic will not bill insurance directly but will provide a superbill for reimbursement, with each appointment costing around \$305–\$310. A referral for metabolic issues was provided for a specialist, Dr. Amina, in Redwood City, and the care team will prioritize the first treatment appointment.

Details

- **Diagnosis and Progression of Vestibular Conditions:** SPEAKER_1's condition involves both central and peripheral vestibular issues, specifically 3PD (central vertigo) and peripheral conditions like Benign Paroxysmal Positional Vertigo (BPPV), along with decreased strength in four out of six semicircular canals. They noted a worsening of



findings since previous tests, including the presence of nystagmus in the dark and gaze-evoked nystagmus in each direction, indicating a progression of the condition.

- **Treatment Timeline and Commitment:** Treatment is estimated to be a complicated process requiring about six months of treatment, with appointments likely occurring twice a week. SPEAKER_1 expressed a willingness to move closer to the clinic in San Francisco if the treatment is expected to be successful and if this clinic is the only effective treatment option. SPEAKER_1 was advised that coming in more frequently might accelerate recovery, provided they feel better and not worse, and the care team will rely heavily on their feedback.
- **Insurance and Billing:** The clinic does not bill insurance companies directly because their modern, contemporary treatment protocols might not be accepted under outdated insurance protocols. The clinic is willing to send a superbill and all records for SPEAKER_1 to seek reimbursement, and the team will assist with any requests from the insurance company to facilitate coverage. The cost for each appointment is consistently around \$305–\$310.
- **Diagnosis Specifics and Symptom Amplification:** When asked to rank the severity of the conditions, the clinician stated that they view the physiology as the same across the separate diagnoses, though they identified spontaneous nystagmus as the most significant finding to eliminate. Regarding Xanax, it was noted that the medication does not cause the symptoms but can amplify them, particularly when tapering off, though the clinician does not specialize in tapering off such medications.
- **Treatment Plan and Safety:** Treatment will follow a tiered approach, beginning with maneuvers to treat BPPV (ear crystals) and focusing on proprioceptive work, followed by slow optinetics using virtual reality to address 3PD, and finally moving into gaze stability drills. While the maneuvers for BPPV can cause temporary dizziness, the team has extensive experience and will proceed cautiously, starting small in the first appointment to ensure the therapy does not make the condition worse, and they rely on SPEAKER_1's feedback to adjust the therapeutic dose.
- **Scheduling and Logistics:** The clinic is open Monday through Friday and does not operate on weekends. SPEAKER_1's case will be prioritized for scheduling, with the team attempting to book the first appointment as early as the next day, if possible, for after 1:00 PM. The team will send a list of potential housing options, including Airbnbs, as SPEAKER_1 is unable to drive and intends to move close to the clinic.
- **Prognosis and Health Recommendations:** The clinician has a high confidence that the treatments will make SPEAKER_1 feel better, though the extent of a full recovery depends on their response to treatment. For general health, an anti-inflammatory diet low in sugar and processed foods, with fresh fruit, vegetables, and good meats, was



recommended, with an option to also try a gluten and dairy-free diet. Insomnia and the vestibular findings are believed to be part of a "vicious cycle," where each condition exacerbates the other.

- **Referrals and Communication:** A referral will be provided for a specialist in Redwood City, Dr. Amina, who deals with metabolic issues, including high cholesterol. The team was instructed to email all requested information, including the report, treatment plan, pricing, superbill, and referral contact, and they will send the recording and transcript of the meeting. The clinician is willing to communicate and send records to any other doctor SPEAKER_1 requests, provided a release of records is signed.